Relative performance change, 2018 → 2026 (positions gained among comparable countries)

Green = better relative standing; red = worse; 0 = same position; – = not comparable. Indicators with a desirable direction only (see Directions). Example: +2 for a mortality row means the country moved 2 places toward the low-mortality end.

Indicator ID	Domain	Indicator	US	UK	Germany	Sweden	France	NLD	CHE	Denmark	Canada	Japan	Australia
F1_05	Spending	Poverty rate, % below poverty line of 60%	0	0	-2	+2	0	+1	+1	0	+1	0	-1
F1_17	Spending	Population with health care coverage, %	0	0	+2	0	0	-2	0	0	0	0	0
F4_01	Population health	Smoking, % of population aged ≥15 y who smoke daily	+1	0	+2	-2	0	+1	-1	0	+2	-2	-1
F4_02	Population health	Alcohol consumption, L per capita in population aged ≥15	-3	+2	-1	-1	+1	0	+4	-1	-1	+1	-1
F4_03	Population health	Obese or overweight, % of population aged ≥15 y	0	-1	–	-1	+3	0	0	-1	0	0	+1
F4_04	Population health	Life expectancy in total population at birth, mean, y	0	-1	0	+2	0	-1	+1	+2	0	-1	-1
F4_05	Population health	Health-adjusted life expectancy, mean, y	0	-2	0	+3	-2	-2	0	+4	-4	0	+3
F4_06	Population health	Life expectancy for women aged ≥40 y, mean, y	0	+1	-1	+1	0	-3	+1	+3	-1	0	0
F4_07	Population health	Life expectancy for men aged ≥40 y, mean, y	0	-1	0	+2	+1	-1	0	+1	0	-2	0
F4_08	Population health	Maternal mortality, deaths per 100 000 live births	0	+1	+5	-3	0	+4	-2	0	-3	+1	-4
F4_09	Population health	Infant mortality, deaths per 1000 live births	0	0	-2	+1	-1	-4	+2	+3	0	0	-3
F4_10	Population health	Neonatal mortality, deaths per 1000 live births	0	-2	-1	0	-2	-1	+2	+5	0	0	-1
F4_11	Population health	Neonatal mortality, deaths per 1000 live births excluding	–	–	–	–	–	–	–	–	–	–	–
F4_12	Population health	Low birth weight, % of total live births	0	+1	+2	0	-5	+3	–	0	-3	0	0
F9_13	Pharmaceuticals	Antibiotic prescribing, defined daily doses per 1000 popul	–	-1	-1	0	0	0	–	-1	+3	–	0
F10_01	Access & quality	Able to get same- or next-day appointment	+1	0	+3	0	-2	0	–	–	0	–	-2
F10_02	Access & quality	2-mo Wait time to see specialist	0	+1	-3	+2	-6	+2	+4	–	0	–	+1
F10_03	Access & quality	Adequate time with regular (primary) physician	+1	-4	-2	+1	–	+2	0	–	-1	–	+3
F10_04	Access & quality	System works well	–	–	–	–	–	–	–	–	–	–	–
F10_05	Access & quality	Fundamental changes needed	–	–	–	–	–	–	–	–	–	–	–
F10_06	Access & quality	Complete rebuild of health system needed	–	–	–	–	–	–	–	–	–	–	–
F10_07	Access & quality	Measles immunization, % of children	+6	-4	-4	-4	+5	-6	0	+6	0	0	-3
F10_08	Access & quality	Breast cancer screening, % of women aged 50-69 y	-1	-1	0	+3	-1	-3	+1	0	+2	0	0
F10_09	Access & quality	30-d Stroke mortality per 1000 patients	-1	-3	-2	+4	-1	–	-4	–	+2	–	+5
F10_10	Access & quality	30-d Mortality per 1000 patients with acute myocardial inf	-2	-2	0	+5	-1	–	0	–	0	–	0
F10_11	Access & quality	Foreign body left per 100 000 discharges	–	–	–	–	–	–	–	–	–	–	–
F10_12	Access & quality	Obstetric trauma without instrument per 100 deliveries	-3	+2	+1	+4	–	-3	+1	-2	0	–	+1
F10_13	Access & quality	Diabetes hospitalizations per 100 000 population	-1	0	+1	+3	0	0	-2	+1	-2	–	0
F10_14	Access & quality	Diabetes hospitalizations as a ratio of population with dia	+1	+2	0	+5	-7	+2	-3	+1	0	–	+2
F10_15	Access & quality	Asthma hospitalizations per 100 000 population	+7	-1	-2	0	-2	+1	-1	-2	0	–	0
F10_16	Access & quality	Asthma hospitalizations as a ratio of population with asth	–	–	–	–	–	–	–	–	–	–	–
F11_01	Equity	Horizontal inequity index, %	0	0	0	–	0	–	–	–	0	–	–
F11_02	Equity	As % of total health expenditure	+1	-5	+3	+3	+1	-3	0	-1	-2	+3	0
F11_03	Equity	As % of household consumption	+1	0	+2	+2	+2	-5	0	-1	+1	+1	-1
F11_04	Equity	Consultation skipped because of cost	0	0	-1	-3	+5	+3	-2	–	-2	–	0
F11_05	Equity	% Unmet need, below-average income	0	-3	-1	-2	+3	+1	+1	–	+1	–	-4
F11_06	Equity	% Unmet need, above-average income	0	-2	-3	0	+3	+4	+1	–	-1	–	-3

Raw rank movement, 2018 → 2026 (all indicators; + = moved toward rank 1 / higher value)

Blue = moved up (higher value relative to others); orange = moved down; – = not comparable. No judgment of good or bad.

Indicator ID	Domain	Indicator	US	UK	Germany	Sweden	France	NLD	CHE	Denmark	Canada	Japan	Australia
F1_01	Spending	Overall population (in millions)	0	0	0	0	0	0	0	0	0	0	0
F1_02	Spending	Population ≥65 y, %	+1	-2	0	-1	+1	+2	-1	0	+1	0	-1
F1_03	Spending	GDP per capita, US $ (in thousands)	+1	+1	+2	-3	-1	+2	0	-2	0	0	0
F1_04	Spending	Land area (× 1000 sq km)	+1	0	0	0	0	-1	0	0	-1	0	0
F1_05	Spending	Poverty rate, % below poverty line of 60%	0	0	+2	-2	0	-1	-1	0	-1	0	+1
F1_06	Spending	Total spending on health, % of total national GDP	0	+3	+2	-3	+1	-2	-1	-4	+4	-2	+2
F1_07	Spending	Public spending on health, % of total national GDP	+2	+2	+1	0	-4	–	–	-6	+2	+2	0
F1_08	Spending	Mean spending on health per capita, US $	0	+1	+3	-3	+1	+1	+1	-4	0	-2	+2
F1_09	Spending	Inpatient care	0	+2	+2	0	-1	–	-3	-4	0	+4	-1
F1_10	Spending	Outpatient care	0	+2	-1	0	+1	–	-4	+2	-1	+1	-1
F1_11	Spending	Long-term care	-1	-1	+3	0	0	–	-1	-2	+5	-4	+1
F1_12	Spending	Medical goods	0	-4	0	+1	-2	–	+2	+1	-3	-1	0
F1_13	Spending	Governance and administration	0	-2	-2	-4	+7	–	0	+2	-2	0	-3
F1_14	Spending	Home-based care	–	0	-1	+1	0	–	–	–	-1	-1	-2
F1_15	Spending	Preventive care	-2	0	-1	-3	+1	–	-2	-5	0	0	-1
F1_16	Spending	Other	0	-1	-2	-1	-2	–	+7	0	-1	-1	-1
F1_17	Spending	Population with health care coverage, %	0	0	+2	0	0	-2	0	0	0	0	0
F4_01	Population health	Smoking, % of population aged ≥15 y who smoke daily	-1	0	-2	+2	0	-1	+1	0	-2	+2	+1
F4_02	Population health	Alcohol consumption, L per capita in population aged ≥15	+3	-2	+1	+1	-1	0	-4	+1	+1	-1	+1
F4_03	Population health	Obese or overweight, % of population aged ≥15 y	0	+1	–	+1	-3	0	0	+1	0	0	-1
F4_04	Population health	Life expectancy in total population at birth, mean, y	0	-1	0	+2	0	-1	+1	+2	0	-1	-1
F4_05	Population health	Health-adjusted life expectancy, mean, y	0	-2	0	+3	-2	-2	0	+4	-4	0	+3
F4_06	Population health	Life expectancy for women aged ≥40 y, mean, y	0	+1	-1	+1	0	-3	+1	+3	-1	0	0
F4_07	Population health	Life expectancy for men aged ≥40 y, mean, y	0	-1	0	+2	+1	-1	0	+1	0	-2	0
F4_08	Population health	Maternal mortality, deaths per 100 000 live births	0	-1	-5	+3	0	-4	+2	0	+3	-1	+4
F4_09	Population health	Infant mortality, deaths per 1000 live births	0	0	+2	-1	+1	+4	-2	-3	0	0	+3
F4_10	Population health	Neonatal mortality, deaths per 1000 live births	0	+2	+1	0	+2	+1	-2	-5	0	0	+1
F4_11	Population health	Neonatal mortality, deaths per 1000 live births excluding	–	–	–	–	–	–	–	–	–	–	–
F4_12	Population health	Low birth weight, % of total live births	0	-1	-2	0	+5	-3	–	0	+3	0	0
F5_01	Workforce & capacity	Overall physicians per 1000 population	-1	+3	+2	0	0	-1	-1	+2	-2	0	0
F5_02	Workforce & capacity	Primary care physicians, % of total	–	–	–	–	–	–	–	–	–	–	–
F5_03	Workforce & capacity	Specialists, % of total	–	–	–	–	–	–	–	–	–	–	–
F5_04	Workforce & capacity	Nurses per 1000 population	+3	+1	-3	-2	-1	-3	0	–	0	+3	+2
F5_05	Workforce & capacity	Generalist physicians	0	+1	0	+1	+2	+1	–	–	-2	-2	-1
F5_06	Workforce & capacity	Specialist physicians	0	0	0	0	-1	0	–	+1	+2	–	-2
F5_07	Workforce & capacity	Nurses	0	-1	+1	–	+2	0	–	0	-1	-1	0
F5_08	Workforce & capacity	Non-health-specific annual wage, mean	-1	+1	+2	+1	-2	0	+1	0	-1	0	-1
F5_09	Workforce & capacity	Ratio of generalist remuneration to mean wage	-2	+1	+1	+1	+1	0	–	–	-1	–	-2
F5_10	Workforce & capacity	Ratio of specialists remuneration to mean wage	0	+4	-4	0	-3	0	–	+1	0	–	0
F5_11	Workforce & capacity	Ratio of nurse remuneration to mean wage	-1	-5	0	–	+1	+1	–	+2	+1	-1	0
F5_12	Workforce & capacity	Magnetic resonance imaging units	0	0	0	–	+2	-1	–	–	0	0	-1
F5_13	Workforce & capacity	Computed tomography units	0	0	+1	–	0	0	+1	-2	0	0	0
F5_14	Workforce & capacity	Mammography machine units	0	-2	–	–	–	–	0	+1	+1	0	0
F5_15	Workforce & capacity	Hospital beds per 1000 population	+1	+1	0	0	0	-3	0	-2	0	0	0
F5_16	Workforce & capacity	Long-term beds per 1000 population aged ≥65 y	0	0	+3	-1	-1	+2	-1	0	-1	0	-1
F7_01	Utilization	Acute myocardial infarction	+4	+2	0	-2	+2	+1	-2	-2	-4	0	+1
F7_02	Utilization	Mental and behavioral	0	+1	0	-2	–	0	0	0	0	-1	+2
F7_03	Utilization	Pneumonia	0	0	+3	-2	0	-1	+2	-2	+1	-4	+3
F7_04	Utilization	Chronic obstructive pulmonary disease	+2	0	0	-3	+1	+1	+1	-1	-1	0	0
F7_05	Utilization	Magnetic resonance imaging	-2	+1	0	–	+2	+1	+1	-1	-2	0	0
F7_06	Utilization	Computed tomography	0	0	0	–	+1	0	0	0	-2	-1	+2
F7_07	Utilization	Total hip replacement per 100 000 population	-2	+1	+1	+1	-1	0	-1	0	0	–	+1
F7_08	Utilization	Total knee replacement per 100 000 population	-5	-1	0	+1	0	0	+3	+2	+1	–	-1
F7_09	Utilization	Hysterectomy per 100 000 women	–	+3	-2	-1	+2	-1	+1	-3	0	–	+1
F7_10	Utilization	Cesarean delivery per 100 live births	-4	+4	-2	0	0	0	-2	-1	+1	–	+2
F7_11	Utilization	Cataract surgery per 100 000 population	–	+1	+1	-1	0	+4	0	+2	-6	–	-2
F7_12	Utilization	Coronary artery bypass graft surgery	0	0	+1	0	+2	-2	–	-5	+3	–	+3
F7_13	Utilization	Coronary angioplasty	–	0	0	-1	+1	-1	–	-1	0	–	+2
F7_14	Utilization	Normal delivery	0	+1	+1	+1	0	0	-1	-2	0	–	0
F7_15	Utilization	Acute myocardial infarction	+3	+1	0	-1	-2	-3	-1	+2	+1	–	0
F9_01	Pharmaceuticals	Total spending per capita, US $	–	–	–	–	–	–	–	–	–	–	–
F9_02	Pharmaceuticals	Retail pharmaceutical spending per capita, US $	0	0	+5	-2	-2	+1	-1	-7	-1	+3	+4
F9_03	Pharmaceuticals	Crestor (cholesterol)	–	–	–	–	–	–	–	–	–	–	–
F9_04	Pharmaceuticals	Lantus (diabetes)	–	–	–	–	–	–	–	–	–	–	–
F9_05	Pharmaceuticals	Advair (asthma)	–	–	–	–	–	–	–	–	–	–	–
F9_06	Pharmaceuticals	Humira (rheumatoid arthritis)	–	–	–	–	–	–	–	–	–	–	–
F9_07	Pharmaceuticals	New chemical entities, No.	–	–	–	–	–	–	–	–	–	–	–
F9_08	Pharmaceuticals	Public spending	–	-1	+1	-1	-1	-1	+5	-1	0	-1	-1
F9_09	Pharmaceuticals	Private insurance	–	-1	-4	-1	+3	-1	-2	0	0	+1	+1
F9_10	Pharmaceuticals	Private out-of-pocket spending	+2	-2	+1	+1	-1	+3	-8	0	+1	+1	+1
F9_11	Pharmaceuticals	Volume	–	-3	+1	-1	-5	+7	-4	0	+1	-2	+4
F9_12	Pharmaceuticals	Value	–	-1	0	-1	-2	+3	+3	+5	-1	-7	-3
F9_13	Pharmaceuticals	Antibiotic prescribing, defined daily doses per 1000 popula	–	+1	+1	0	0	0	–	+1	-3	–	0
F10_01	Access & quality	Able to get same- or next-day appointment	+1	0	+3	0	-2	0	–	–	0	–	-2
F10_02	Access & quality	2-mo Wait time to see specialist	0	-1	+3	-2	+6	-2	-4	–	0	–	-1
F10_03	Access & quality	Adequate time with regular (primary) physician	+1	-4	-2	+1	–	+2	0	–	-1	–	+3
F10_04	Access & quality	System works well	–	–	–	–	–	–	–	–	–	–	–
F10_05	Access & quality	Fundamental changes needed	–	–	–	–	–	–	–	–	–	–	–
F10_06	Access & quality	Complete rebuild of health system needed	–	–	–	–	–	–	–	–	–	–	–
F10_07	Access & quality	Measles immunization, % of children	+6	-4	-4	-4	+5	-6	0	+6	0	0	-3
F10_08	Access & quality	Breast cancer screening, % of women aged 50-69 y	-1	-1	0	+3	-1	-3	+1	0	+2	0	0
F10_09	Access & quality	30-d Stroke mortality per 1000 patients	+1	+3	+2	-4	+1	–	+4	–	-2	–	-5
F10_10	Access & quality	30-d Mortality per 1000 patients with acute myocardial infa	+2	+2	0	-5	+1	–	0	–	0	–	0
F10_11	Access & quality	Foreign body left per 100 000 discharges	–	–	–	–	–	–	–	–	–	–	–
F10_12	Access & quality	Obstetric trauma without instrument per 100 deliveries	+3	-2	-1	-4	–	+3	-1	+2	0	–	-1
F10_13	Access & quality	Diabetes hospitalizations per 100 000 population	+1	0	-1	-3	0	0	+2	-1	+2	–	0
F10_14	Access & quality	Diabetes hospitalizations as a ratio of population with dia	-1	-2	0	-5	+7	-2	+3	-1	0	–	-2
F10_15	Access & quality	Asthma hospitalizations per 100 000 population	-7	+1	+2	0	+2	-1	+1	+2	0	–	0
F10_16	Access & quality	Asthma hospitalizations as a ratio of population with asth	–	–	–	–	–	–	–	–	–	–	–
F11_01	Equity	Horizontal inequity index, %	0	0	0	–	0	–	–	–	0	–	–
F11_02	Equity	As % of total health expenditure	-1	+5	-3	-3	-1	+3	0	+1	+2	-3	0
F11_03	Equity	As % of household consumption	-1	0	-2	-2	-2	+5	0	+1	-1	-1	+1
F11_04	Equity	Consultation skipped because of cost	0	0	+1	+3	-5	-3	+2	–	+2	–	0
F11_05	Equity	% Unmet need, below-average income	0	+3	+1	+2	-3	-1	-1	–	-1	–	+4
F11_06	Equity	% Unmet need, above-average income	0	+2	+3	0	-3	-4	-1	–	+1	–	+3
F11_07	Equity	Rural population, % of total population	0	0	-1	-2	0	–	–	0	-2	0	+2
F11_08	Equity	Population density per sq mile	0	0	0	0	0	0	0	0	0	0	0
F11_09	Equity	Urban physicians per 1000 population	–	–	–	–	–	–	–	–	–	–	–
F11_10	Equity	Rural physicians per 1000 population	–	–	–	–	–	–	–	–	–	–	–